

RIVERSIDE GENERAL HOSPITAL

500 Medical Center Drive, Riverside, CA 92501
Emergency Department: (951) 555-7000

EMERGENCY DEPARTMENT RECORD

PATIENT INFORMATION

Patient Name:	Maria Elena Santos	MRN:	RGH-2024-081547
DOB:	03/14/1988	Age:	35 years
Sex:	Female	Weight:	142 lbs
Address:	2847 Maple Grove Dr, Apt 12, Riverside, CA 92501		
Phone:	(951) 555-0147	Insurance:	Hartford WC

VISIT INFORMATION

Date of Visit:	February 8, 2024	Arrival Time:	10:42 AM
Mode of Arrival:	EMS/Ambulance	Triage Level:	2 - Emergent
Attending MD:	Dr. Jennifer Walsh	Discharge Time:	4:35 PM

CHIEF COMPLAINT

Fall from height (approximately 15 feet) at workplace - right wrist pain, head injury, back pain.

HISTORY OF PRESENT ILLNESS

35-year-old female construction worker brought in by EMS following a fall from scaffolding at a construction site. Per EMS report, patient fell approximately 15 feet onto concrete floor. Patient was conscious at scene with GCS 14. Patient complains of severe right wrist pain (10/10), headache (7/10), and lower back pain (8/10). Patient denies loss of consciousness but reports feeling “dizzy” immediately after the fall. No neck pain. No numbness or tingling in extremities.

PHYSICAL EXAMINATION

Vital Signs on Arrival:

- BP: 148/92 mmHg
- HR: 98 bpm
- RR: 18/min
- Temp: 98.4°F
- SpO2: 99% on room air
- Pain: 8/10

General: Alert, oriented x3, in moderate distress due to pain.

HEENT: 3cm laceration to right forehead, no active bleeding (controlled at scene). Pupils equal, round, reactive to light. No Battle’s sign. No hemotympanum.

Neck: C-spine immobilized per protocol. No midline tenderness on palpation. Full ROM after cleared.

Chest: Clear to auscultation bilaterally. No chest wall tenderness.

Cardiovascular: Regular rate and rhythm, no murmurs.

Abdomen: Soft, non-tender, non-distended.

Back: Tenderness to palpation over L4-L5 region. Paraspinal muscle spasm noted. No step-off deformity.

Extremities:

- Right upper extremity: Obvious deformity of distal radius. Significant swelling and tenderness. Unable to move wrist. Radial pulse intact. Sensation intact to light touch in all digits.
- Left upper extremity: Abrasion to elbow, ROM intact.
- Lower extremities: Contusion to left hip, ROM intact bilaterally. Pulses intact.

Neurological: GCS 15. Cranial nerves II-XII intact. Motor strength 5/5 all extremities except limited by pain in RUE. Sensation intact.

DIAGNOSTIC STUDIES

Imaging:

- **X-ray, Right Wrist:** Displaced fracture of the distal radius with dorsal angulation. Ulnar styloid intact.
- **CT Head without contrast:** No acute intracranial hemorrhage. No skull fracture. No midline shift.
- **X-ray, Lumbar Spine:** No acute fracture. Mild degenerative changes at L4-L5.
- **CT Cervical Spine:** No acute fracture or dislocation.

Laboratory:

- CBC: WNL
- BMP: WNL
- Coagulation panel: WNL

ASSESSMENT / DIAGNOSES

1. Closed displaced fracture, distal radius, right (S52.501A)
2. Concussion without loss of consciousness (S06.0X0A)
3. Lumbar strain (S39.012A)
4. Contusion, left hip (S70.01XA)
5. Laceration, forehead, 3cm (S01.81XA)

TREATMENT PROVIDED

1. IV access established, normal saline initiated
2. Pain management: Morphine 4mg IV x2
3. Forehead laceration cleaned and closed with 6 sutures (4-0 nylon)
4. Right wrist: Closed reduction performed under conscious sedation. Post-reduction X-ray confirms acceptable alignment. Splinted in sugar-tong splint.
5. Tetanus booster administered
6. Ice packs applied to lumbar region and left hip

DISPOSITION

Patient **discharged home** in stable condition with the following:

Discharge Instructions:

- Concussion precautions provided (written and verbal)
- Keep splint clean and dry
- Elevate right arm above heart level when possible
- Ice lumbar area 20 min every 2 hours while awake
- Return immediately for: worsening headache, vomiting, vision changes, numbness/tingling, increased pain, fever

Medications Prescribed:

- Norco 5/325mg - 1 tablet every 6 hours as needed for pain, #30
- Ibuprofen 600mg - 1 tablet every 8 hours with food, #30
- Flexeril 10mg - 1 tablet at bedtime for muscle spasm, #14

Follow-up:

- Orthopedic Surgery within 2 days (referral provided)
- Primary care within 1 week
- Return to ED if symptoms worsen

Jennifer Walsh, MD
Emergency Medicine
Riverside General Hospital

Date/Time: 02/08/2024 16:35